



NATIONAL ASSOCIATION OF
CHAIN DRUG STORES

February 23, 2026

Dr. Thomas Keane

Assistant Secretary for Technology Policy / Office of the National Coordinator for Health Information
Technology

Attention: Request for Information: HHS Health Sector AI RFI

Mary E. Switzer Building

330 C Street, S.W.

Washington, D.C. 20201

Submitted via: www.regulations.gov

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of
Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane,

The National Association of Chain Drug Stores (NACDS) appreciates the opportunity to respond to the Request for Information (RFI) on accelerating the adoption and responsible use of artificial intelligence (AI) in clinical care. We applaud your commitment to advancing the appropriate use of AI to improve healthcare for the American people and appreciate your consideration of our comments. NACDS represents chain pharmacies, including traditional drug stores, supermarkets, and mass merchants with pharmacies. Nearly 90% of Americans live within 5 miles of a community pharmacy¹ and 85% of adults in the U.S. say pharmacists are easy to access, ranking them the most accessible healthcare destination.² Pharmacies are a trusted, nationwide access point for medications, immunizations, preventive care, patient education and counseling, and a growing range of health and wellness services that improve health outcomes.

To accelerate AI adoption in clinical care, NACDS recommends a pro-innovation, risk proportionate national framework that maintains patient safety, privacy, transparency, equity, and public trust. In implementing AI policy, HHS should rely on and reinforce existing guardrails, avoid duplicative regulation, align reimbursement with value, and ensure harmonization across federal agencies and states. Furthermore, payment reform is foundational. AI should support clinician-rendered care, and clinicians must be reimbursed for that care.

NACDS Recommendations

1. Establish a Harmonized, Risk-Proportionate Federal AI Framework

HHS should rely on and reinforce existing regulatory guardrails, across FDA, CMS, OCR, and ASTP/ONC, rather than layering duplicative AI-specific policies. Interagency coordination and federal

¹ Berenbrok L, Tang S, et al. Access to community pharmacies: A nationwide geographic information systems cross-sectional analysis. JAPhA. July 2022. [https://www.japha.org/article/S1544-3191\(22\)00233-3/fulltext](https://www.japha.org/article/S1544-3191(22)00233-3/fulltext)

² Polling data Conducted by Morning Consult (Commissioned by NACDS). October 2023. <https://www.nacds.org/pdfs/Opinion-Research/NACDS-OpinionResearch-National.pdf>

harmonization are essential to avoid fragmented oversight and inconsistent state-level requirements that create uncertainty and slow responsible innovation. A unified national framework will provide clarity, reduce compliance friction, and support scalable AI deployment in clinical care settings, including community pharmacies.

2. Align Payment Policy with AI-Supported Clinical Value & Recognize Pharmacists as Providers in Medicare Part B

Sustainable adoption of AI in clinical care depends on reimbursement reform. Pharmacists must be recognized as providers under Medicare Part B so that pharmacist-rendered clinical services, including those supported and scaled by AI, are eligible for reimbursement. Without provider recognition by CMS, payment pathways remain limited and investment in AI-enabled clinical services is constrained for pharmacies.

Reimbursement methodologies must also account for the cost of acquiring, implementing, maintaining, and monitoring AI tools that support clinical care delivery. Just as payment rates reflect the resources required to furnish services, reimbursement for AI-supported services should incorporate the reasonable and necessary costs of technology infrastructure, system integration, validation, cybersecurity safeguards, staff training, and ongoing performance monitoring. Without recognition of these costs in rate-setting methodologies, providers may be financially disincentivized from adopting innovative, AI-driven tools that improve quality, safety, and efficiency.

3. Strengthen Interoperability to Enable Authorized, Workflow-Integrated Data Access

Effective clinical AI tools require reliable, structured access to electronic health information within workflow. HHS should strengthen interoperability to ensure that community pharmacies can obtain and transmit clinically relevant data including screening and lab results, discharge summaries, diagnoses, medication changes, care plans, and other key information in real time. Meaningful interoperability will widen market opportunities, promote competition among developers, and enable safe, effective AI integration.

4. Provide Clear Governance Guardrails for Non-Device AI

HHS should issue risk-tiered guidance clarifying documentation standards, liability allocation, oversight expectations, privacy and data governance responsibilities, and audit treatment for AI tools that support, but do not replace, clinical judgment. Safe harbors for good-faith adoption of AI tools would reduce uncertainty and encourage responsible innovation while preserving accountability and public trust.

5. Invest in Applied Research and Real-World Demonstrations

HHS should prioritize public-private partnerships and demonstration projects that validate AI-supported clinical tools in real-world pharmacy settings.

Introductory Perspectives: A National Framework, Payment Alignment, and R&D Investment

1. The Impact of Current HHS Regulations on AI Adoption

HHS currently regulates many aspects of clinical AI use through existing frameworks, including FDA oversight for software as a medical device where applicable; CMS program integrity and

documentation rules; HIPAA privacy and security regulations; Section 1557 nondiscrimination requirements; and ASTP/ONC transparency and risk management requirements for predictive decision support in certified health IT.

These guardrails provide meaningful patient protections and serve as a strong foundation for responsible AI deployment. However, fragmentation and uncertainty across regulatory regimes can slow AI adoption, particularly for non-device AI tools that support clinical documentation, medication adherence outreach, and workflow prioritization. When expectations for oversight, documentation, and audit treatment are unclear or inconsistently interpreted, providers are hesitant to deploy AI tools even when those tools enhance quality and efficiency.

NACDS recommends that HHS rely on existing oversight structures rather than layering duplicative AI-specific policies. Interagency coordination across FDA, CMS, OCR, and ASTP/ONC is essential to avoid conflicting expectations and duplicative compliance burdens. Additionally, HHS should establish a harmonized national framework that minimizes inconsistent state-level AI mandates. In particular, the proliferation of overlapping state AI laws creates uncertainty and inefficiency for healthcare providers operating across multiple jurisdictions. A coordinated federal framework that supersedes state laws would reduce friction and enable responsible scaling of AI tools.

2. Payment Policy Changes to Incentivize High-Value AI and Promote Competition

Payment policy is a primary determinant of AI adoption. Even when AI tools improve quality, efficiency, and access, providers frequently lack sustainable reimbursement pathways. In pharmacy settings, this challenge is amplified by the lack of pharmacist recognition as healthcare providers in Medicare Part B, limiting billing pathways for pharmacist-led clinical services, and uncertainty regarding documentation standards when AI supports clinical workflows.

First, HHS should recognize and reimburse pharmacist-rendered services that AI helps scale. AI should support pharmacist-led screening programs, adherence interventions, chronic care management, preventive services, and other care. However, these services must be reimbursable to achieve widespread adoption. Importantly, reimbursement must reflect not only the professional service furnished but also the technology investments required to deliver AI-supported care. AI tools involve upfront capital expenditures, integration costs, cybersecurity protections, ongoing licensing fees, and continuous validation and monitoring. Payment policy should therefore incorporate technology enablement costs into rate-setting methodologies and value-based payment calculations. Without payment reform, AI will remain limited to administrative efficiencies rather than enabling scalable clinical transformation.

Second, CMS should clarify billing and audit expectations for AI-assisted documentation. When a licensed clinician reviews, edits as needed, and attests to documentation supported by AI, that documentation should be treated consistently with traditional records. Clear guidance would reduce audit uncertainty and encourage adoption of AI tools that reduce clinician burden.

Third, value-based payment models should reward prevention, adherence, and early intervention. Pharmacists play a critical role in delivering these services through medication therapy management, chronic disease monitoring, immunizations, test-and-treat protocols, targeted medication adherence

interventions, and others, often serving as the most accessible healthcare professional in many communities. AI tools that enhance pharmacists' ability to identify high-risk patients, close therapy gaps, support transitions of care, proactively manage chronic conditions, and other care services, can reduce avoidable hospitalizations and improve outcomes. These pharmacist-led, AI-supported services should be financially supported, including through outcome-based payment structures that recognize and reimburse measurable improvements in quality and total cost of care.

Finally, scalable reimbursement pathways, including demonstration programs and shared-savings arrangements, can lower adoption barriers, particularly in rural and underserved communities where pharmacies are often the most accessible healthcare providers.

3. HHS Investment in Research, Development, and Long-Term Market Creation

HHS can catalyze durable, high-value AI markets by investing in public-private partnerships that validate AI-enabled clinical tools in real-world pharmacy settings. Demonstration projects in community pharmacies can generate real-world evidence regarding safety, effectiveness, workflow integration, and total cost of care.

Responses to Specific RFI Questions

RFI Question 1. Biggest Barriers to Private Sector Innovation in AI For Health Care and its Adoption/Use in Clinical Care

The biggest barriers to private sector innovation and adoption are misaligned incentives, regulatory uncertainty, and data access friction, particularly for non-device AI that supports workflow optimization and documentation.

Payment misalignment remains the most significant barrier. Many high-value AI-enabled interventions, including screening and referral workflows, adherence targeting, and chronic care support do not have dependable reimbursement pathways. In pharmacy settings, the lack of consistent payment for pharmacist-rendered clinical services, including in Medicare Part B, suppresses investment in AI tools designed to scale those services, and further constrains innovation. Pharmacists' lack of recognition as providers under Medicare Part B restricts reimbursement eligibility and weakens the market signals necessary to drive AI deployment in pharmacy-based care models.

Fragmented interoperability also limits AI effectiveness. AI systems depend on timely, structured access to relevant clinical data, such as laboratory results, discharge summaries, updated diagnoses, and medication changes. Without reliable real-time data access, AI tools cannot safely or effectively support clinical decision-making.

Finally, fragmented governance frameworks and overlapping federal and state requirements create compliance complexity that slows innovation, particularly for organizations operating across multiple jurisdictions.

RFI Question 2. Regulatory, Payment Policy, or Programmatic Design Changes HHS Should Prioritize

HHS should prioritize payment and program design reforms that make high-value AI-supported clinical services financially viable, paired with clearer guardrails for documentation and audit treatment and strengthened interoperability.

A. Payment and Program Design Priorities (CMS)

HHS should create and expand pathways that reimburse pharmacist-rendered clinical services supported by AI. Without payment reform, AI will remain confined to “back office” functions such as Interactive Voice Response (IVR) optimization, intake triage, and billing support. Existing CMS authorities, including demonstration programs and value-based models, should be used to support pharmacist-led, AI-supported clinical services, such as screening and preventive care, medication adherence interventions, transitions-of-care services, and chronic disease management.

CMS should also clarify billing and documentation expectations when AI supports clinical documentation. If a licensed clinician, including a pharmacist, reviews and attests to the documentation record, AI assistance should not create noncompliance or heightened audit risk.

B. Interoperability and Information Access Priorities (ASTP/ONC)

HHS and ASTP/ONC should strengthen meaningful access to electronic health information (EHI) for all care settings, including community pharmacies. Pharmacies must be able to access clinically relevant EHI in structured formats integrated into workflow, not merely through document-based exchange. Reliable interoperability will expand market opportunity, improve competition among AI developers, and support safe, scalable adoption. NACDS looks forward to working with HHS and ASTP/ONC to strengthen pharmacy interoperability and ensure that community pharmacies are fully integrated participants in national health information exchange infrastructure. As recognized in *ONC’s 2023 Pharmacy Interoperability and Emerging Therapeutics Task Force Report*, pharmacies must be fully integrated into nationwide health information exchange infrastructure to support clinical services through secure, structured data exchange.³

RFI Question 3. Novel Legal/Implementation Issues for Non-Medical Device AI and HHS’ Role

Non-device AI tools raise legal and governance issues not fully addressed by traditional medical device frameworks.

Liability allocation is a central concern impeding adoption of clinical AI tools. When AI suggests prioritization or drafts documentation, responsibility for potential errors must be clearly delineated among clinicians, provider organizations, and AI vendors. Clear expectations are needed regarding standards of clinician review, documentation, and appropriate logging practices.

Privacy, security, and data governance considerations are also significant. Handling electronic health information at scale for AI requires clear expectations for access controls, retention policies, model

³ Office of the National Coordinator for Health Information Technology (ONC), *Pharmacy Interoperability and Emerging Therapeutics Task Force 2023 Recommendations Report*, Health Information Technology Advisory Committee (HITAC), November 9, 2023.

training boundaries, and incident response procedures, while ensuring requirements are not overly burdensome.

HHS should publish risk-tiered guardrails for non-device AI, clarify documentation and oversight expectations, and promote safe harbors for good-faith adoption of AI tools. Specifically, oversight expectations should be proportionate to the level of clinical risk posed by the AI tool. Lower-risk tools that support administrative functions should not face the same regulatory burden as higher-risk tools that influence treatment decisions. A risk-based framework would provide clarity, protect patients, and allow responsible innovation to advance without unnecessary barriers. Encouraging standardized contractual best practices addressing vendor accountability and shared responsibility would further reduce uncertainty.

RFI Question 7. Key Decision Makers/Governing Bodies and Primary Administrative Hurdles

Key Decision Makers: Adoption of AI in clinical care settings, including community pharmacy, is rarely driven by a single individual within an organization. Instead, it reflects coordinated decision-making across multiple leadership domains, each of which evaluates risk, value, and operational feasibility through a distinct lens.

Clinical leadership, including pharmacy leadership and quality committees, plays a central role in determining whether an AI tool aligns with patient safety standards, clinical workflows, and quality improvement goals. These leaders assess whether the technology meaningfully enhances care, reduces errors, improves health outcomes, or supports access. Clinical leaders are often highly influential voices in determining whether AI is perceived as a tool that augments professional judgment or introduces unacceptable risk.

Compliance, legal, and risk management teams also exercise substantial influence. These stakeholders evaluate regulatory exposure, documentation standards, audit risk, liability allocation, and consistency with federal and state requirements. If expectations regarding oversight, documentation, or audit treatment are unclear or overly complex, compliance leadership may appropriately take a cautious posture that slows adoption.

Information technology, security, and data governance leadership are equally critical decisionmakers. AI tools require secure integration with existing pharmacy management systems, access to electronic health information, and adherence to cybersecurity standards. IT leaders must assess data architecture, Application Programming Interface (API) capabilities, access controls, model monitoring requirements, and long-term system maintenance obligations. Concerns regarding data security, privacy compliance, and operational resilience can significantly affect implementation timelines.

Finance and reimbursement leadership evaluate the return on investment, sustainability, and payment viability of AI-supported services. Even when clinical and operational leaders support adoption, uncertainty regarding reimbursement pathways, billing compliance, and audit posture may constrain investment decisions. In pharmacy practice, the absence of consistent payment mechanisms for pharmacist-rendered clinical services further limits financial justification for deploying AI tools designed to scale those services.

Primary Administrative Hurdles: Vendor due diligence is among the most significant barriers. Healthcare organizations must evaluate cybersecurity safeguards, HIPAA compliance, data minimization practices, bias testing methodologies, model validation processes, and ongoing monitoring plans. This process is resource-intensive and often requires multidisciplinary review. Contracting and indemnification complexity further complicates adoption. AI vendor agreements frequently involve negotiations over liability allocation, performance guarantees, data ownership, audit rights, and incident response responsibilities. Providers are often reluctant to accept disproportionate liability exposure for tools they do not control, particularly where regulatory standards are still evolving.

Integration burden with existing systems presents another substantial challenge. AI tools must function within high-volume, workflow-driven environments such as community pharmacies. Poorly integrated systems can increase cognitive load, create alert fatigue, or require duplicative documentation. Seamless interoperability with pharmacy management systems and external electronic health record platforms is essential but not always readily achievable.

Finally, unclear reimbursement and audit posture remains a critical barrier. Organizations need confidence that AI-assisted documentation and clinical decision support will be treated consistently under program integrity reviews. Ambiguity in billing standards or audit expectations creates risk aversion and delays adoption, even when the technology demonstrates clinical value.

Reducing these administrative hurdles through clearer guidance, aligned payment policy, standardized evaluation frameworks, and predictable audit treatment would significantly accelerate responsible AI adoption in clinical care settings.

RFI Question 8. Where Enhanced Interoperability Would Widen Market Opportunities and Accelerate AI Development

Interoperability improvements that matter most for pharmacy-enabled AI include structured access to relevant clinical information, such as medication history, laboratory data, diagnoses, discharge summaries, care plans, and referral documentation within workflow.

If pharmacies can reliably access electronic health information in usable formats integrated into daily workflow, the AI market expands dramatically. More developers can compete, more clinical use cases become viable, and patient outcomes improve through safer, more efficient, and better coordinated care.

Conclusion

Pharmacies are uniquely positioned to expand access to a wide variety of healthcare services through responsibly deployed clinical AI tools. However, meaningful and scalable adoption requires payment reform, documentation clarity, strengthened interoperability, and risk-proportionate governance guardrails. NACDS stands ready to collaborate with HHS and across agencies to ensure that AI enhances clinical care in ways that are safe, responsible, competitive, and scalable nationwide. NACDS is eager to collaborate to improve pharmacy data interoperability and ensure community pharmacies are completely included in the nation's

health information exchange system. For any questions or further discussion regarding these comments, please contact Kayla McFeely, Vice President, Health & Wellness Strategy and Policy at kmcfeely@nacds.org.

Sincerely,

A handwritten signature in black ink, appearing to read "Steven C. Anderson". The signature is fluid and cursive, with a long horizontal stroke at the end.

Steven C. Anderson, FASAE, CAE, IOM
President and Chief Executive Officer
National Association of Chain Drug Stores

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NACDS represents traditional drug stores, supermarkets and mass merchants with pharmacies. Chains operate nearly 40,000 pharmacies, and NACDS' chain member companies include regional chains, with a minimum of four stores, and national companies. Chains employ nearly 3 million individuals, including 155,000 pharmacists. They fill over 3 billion prescriptions yearly, and help patients use medicines correctly and safely, while offering innovative services that improve patient health and healthcare affordability. NACDS members also include more than 900 supplier partners and over 70 international members representing 21 countries. Please visit NACDS.org.